كلية التمريض
٢٠٠٧

جامعة حلوان

# OCD

## Goals:

To provide the participants with knowledge, skills and positive attitude toward anxiety disorders.

## Objectives:-

**At the end of this presentation student will be able to**

-Define fear, stress, anxiety and anxiety disorder.

-Differentiate between anxiety and anxiety disorders.

-Identify etiology of anxiety disorders.

- Clarify the types of anxiety disorders and signs & symptoms of anxiety.

- Apply line of treatment and nursing management for anxiety.

- Gain positive attitude toward anxiety disorders.

## Introduction

Experiencing occasional anxiety is a normal part of life. However, people with anxiety disorders frequently have intense, excessive and persistent worry and fear about everyday situations. Often, anxiety disorders involve repeated episodes of sudden feelings of intense anxiety and fear or terror that reach a peak within minutes in some cases (panic attack).

# Obsessive compulsive disorder (OCD)

## Definition

**Obsessive compulsive disorder (OCD):** This disorder is characterized by obsessions and compulsions.

**Obsessions**: recurrent and persistent thoughts, urges, or images; experienced as intrusive and unwanted.

**Compulsions:** repetitive behaviors or mental acts that an individual feels compelled to perform in response to obsessions or rules

## Causes of Obsessive compulsive disorder (OCD):

1. Genetic factors.( Having blood relatives with an anxiety disorder)
2. Neuropsychiatric disorder.
3. Environmental stress.
4. Substance abuse.
5. Mental health conditions.
6. Medical conditions.
7. Childhood experience.
8. Current life situation.
9. Social psychological factors.

## Signs and symptoms

1- Severe anxiety, worry, or distress.
2- Avoidant behavior
3- Excessive time spent on parts of daily routine, that is, showering, cleaning
4- Obsessions and compulsions
5- Odd or excessive behaviors
6- Significant impairment (i.e., patient is unable to work/attend school or participate in social activities).

**Obsessions are defined by the following:**

- Recurrent thoughts, urges, or images are experienced as intrusive and unwanted, < and in most people cause marked distress or anxiety.
- The individual makes attempts to ignore or suppress the offending thoughts, urges, or images, or to neutralize them with other thoughts and/ or activities.

**Compulsions are defined by**

- Repetitive behaviors (e.g., handwashing) or repetitive mental activities (e.g., counting items, repeating words silently)
- The individual feels driven to perform these repetitive behaviors or mental activities

- Children may or may not be able to explain their behaviors or mental activities.
- The obsessions or compulsions cause distress for the individual, are time consuming, or significantly interfere with daily routine.

**Recommended therapy:**

- The two main treatments for OCD include psychotherapy and medication.
- **Exposure and response prevention (ERP) and CBT** offer the greatest benefits, or efficacy and specificity, in treating the symptoms.
- **ERP** involves gradually exposing person to a feared object or obsession, such as something dirty, while teaching the patient ways to cope with the resulting anxiety.
- **CBT** involves retraining thought patterns and routines so that compulsive behaviors are no longer needed to relieve anxiety.

- **Selective-serotonin reuptake inhibitors (SSRIs):**First-line drug therapy:
  - Fluoxetine (Prozac)
  - Sertraline (Zoloft)
- **Tricyclic antidepressants (TCAs) :** Drugs for treatment-resistant cases
  - Clomipramine (Anafranil)

**<u>Risk factors of OCD include:</u>**

1-**Genetics** and **Family History**: Having a family member with an anxiety disorder increases the likelihood of developing one due to genetic predisposition.

**2-Personality Traits:** Traits such as being prone to negative thoughts, being shy, or being withdrawn can elevate the risk.

**3-Trauma** and **Stress**: Experiencing significant stress or trauma, especially during childhood, can trigger anxiety disorders. This includes abuse, neglect, and the death of a loved one.

**4-Health Conditions**: Chronic physical conditions like heart disease, diabetes, respiratory disorders, and chronic pain can be linked to anxiety disorders.

**5-Substance Abuse**: Abuse of alcohol, drugs, or caffeine can exacerbate or trigger anxiety symptoms.

**6-Brain Chemistry:** Imbalances in neurotransmitters, which are chemicals in the brain that help regulate mood and behavior, can play a significant role.

**7-Chronic Stress:** Persistent stress from work, relationships, or other aspects of life can contribute to the development of anxiety disorders.

**8-Cultural Factors**: Societal and cultural expectations and pressures can influence anxiety, with some cultures placing higher stress on certain behaviors and achievements.

**9-Gender**: Women are more likely than men to develop anxiety disorders, possibly due to hormonal differences and societal expectations.

**10-Lack of Social Support:** A lack of a supportive social network can increase the risk, as social connections can act as a buffer against stress.

## Line of treatment:

**A. Hospitalization:**

1. In case of Suicidal ideation or homicidal hospitalization is required.
2. Person experience sever symptoms enough to impair their ability to function at work and home.
3. Person require high level of care.

**B. Psychotherapy**:

- **Cognitive Behavioral Therapy:** is the most widely used therapy for anxiety disorders. Research has found it to be effective in treating SAD, GAD, phobias, and panic disorders, among other conditions.
- **Dialectical behavior therapy (DBT):** is a highly effective type of CBT. Originally used to treat borderline personality disorder (BPD), DBT is now used to treat a variety of conditions, including anxiety.

**C. Pharmacological**:

Antidepressant agents are the drugs of choice in the treatment of anxiety disorders:

**1. Selective serotonin reuptake inhibitors:** The SSRIs are first-line agents for long-term management of anxiety disorders.

**2. Serotonin and norepinephrine reuptake inhibitors:** Pharmacologic agents with reuptake inhibition of serotonin and norepinephrine may be helpful in a variety of mood and anxiety disorders.

**3. Norepinephrine and serotonin:** while also blocking some postsynaptic serotonergic receptors that conceptually mediate excessive anxiety when stimulated with serotonin.

**4. Tricyclic antidepressants:** are a complex group of drugs that have central and peripheral anticholinergic effects, as well as sedative effects.

**Nursing Management**

• Assess the intensity of anxiety

• Determine the triggers for anxiety

• Assess how the patient responds to anxiety

• Administer medications to relieve anxiety

• Educate the patient about anxiety

• Encourage patient to develop support groups

• Encourage patient to seek mental health counseling

• Educate patient on self-care

• Provide means of support

• Interact with the patient in a calm and gentle manner

• Converse in simple language

• Allow the patient to talk about distressing emotions and feelings

• Assess patient for suicidal ideations

• Help strengthen patient's problem-solving abilities

• Tell the patient to limit alcohol and caffeinated beverages

• Encourage patient to participate in social functions

## References


- Ahmari, S. E., Spellman, T., Douglass, N. L., Kheirbek, M. A., Simpson, H. B., Deisseroth, K., & Hen, R. (2013). Repeated cortico-striatal stimulation generates persistent OCD like behavior. Science, 340(6137), 1234–1239.
- American Psychiatric Association. (2000). Diagnostic and statistical manual of mental disorders (4th ed., text rev.). American Psychiatric Association.
- American Psychiatric Association. (2013). Diagnostic and statistical manual of mental disorders (5th ed.). American Psychiatric Publishing.
- Gibbons, R. D., Brown, C. H., Hur, K., Davis, J., & Mann, J. J. (2012). Suicidal thoughts and behavior with antidepressant treatment: Reanalysis of the randomized placebo controlled studies of fluoxetine and venlafaxine. Archives of General Psychiatry, 69(6), 580–587.
- Goodman, W. K., Price, L. H., Rasmussen, S. A., Mazure, C., Fleischmann, R. L., Hill, C. L., ... Charney, D. S. (1989). The Yale-Brown obsessive compulsive scale: Pt. I. Development, use, and reliability. Archives of General Psychiatry, 46(10), 1006–1011.
- Stein, D. J., Koen, N., Fineberg, N., Fontenelle, L. F., Matsunaga, H., Osser, D., & Simpson, H. B. (2012). A 2012 evidence-based algorithm for the pharmacotherapy for obsessive-compulsive disorder. Current Psychiatry Reports, 14(3), 211–219. https://doi.org/10.1007/s11920–012-0268–9
- World Health Organization. (2001). The World Health report 2001—Mental health: New understanding, new hope. World Health Organization.
- Wu, K., Hanna, G. L., Rosenberg, D. R., & Arnold, P. D. (2012). The role of glutamate signaling in the pathogenesis and treatment of obsessive-compulsive disorder. Pharmacology, Biochemistry, and Behavior, 100(4), 726–735. https://doi.org/10.1016/j.pbb.2011.10.007